TEAM CASE STUDY: RAY KESSLER

The whole class is ONE nursing team for one patient across 24 hours. We move through the time together, 4 hours at a time.

5 East, Room 14 | Tue 0700 to Wed 0700

Write your team name in the header at the top of this page (it repeats on every page).

Name (print)	Letter	Initials
	A	
	B	
	C	
	D	
	E	
	F	
	G	

HOW THE TWO HOURS WORK

- The instructor moves the clock forward **4 hours at a time**: 0700, 1100, 1500, 1900, 2300, 0300, and a final check at 0700.
- At each checkpoint, the instructor reads the situation. The team looks at Ray and the chart. The **charge nurse asks every nurse for one thing** before deciding, and **only the charge nurse writes** in the shared boxes while the others advise. The charge nurse makes one SBAR call.
- The packet is shared by the table; each student has their own reflection page (last page). You finish each page at the **end** of its time block. **Do not turn the page** until the instructor advances the clock.
- Use the EHR on your own device (vitals, meds, results). Your patient and the time are synced with the instructor. Write in dark pen.

ROLES

CHARGE NURSE (one person; changes every checkpoint)	Coordinates and organizes the team. Keeps time. Decides who looks at what. Leads a quick huddle. Decides the priorities with the team. **Makes the SBAR call and writes down the team plan.**
NURSE (everyone else)	You are the eyes and the thinking. Look at Ray, the chart and the monitor, share what you find, and speak up. At each checkpoint the charge nurse and the team choose a focus. For example, one nurse looks at meds and treatments and one at results and the monitor. Then speak up when the charge nurse asks.

CHARGE NURSE ROTATION (each student is charge nurse exactly once)

Stop	0700	1100	1500	1900	2300	0300	Close 0700
Student letter	A	B	C	D	E	F	G
Name / initials							

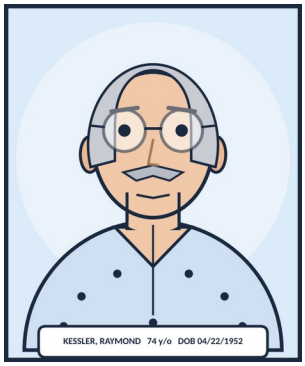
Smaller teams: with 4 people the letters go A B C D A B C; with 3, A B C A B C A. The charge nurse organizes; the whole team still does the thinking.

TURN-IN

One shared packet for the team, fastened with a **paperclip (no staples)**. The last page has an individual reflection and a peer check: write your own name on yours.

PATIENT CARD: RAYMOND "RAY" KESSLER

Room 5E-14



Name: Raymond "Ray" Kessler, he/him, 74 years old, DOB 04/22/1952

MRN: SIM-40318

Height / weight: 175 cm | 88.0 kg on arrival (usual weight about 90 kg)

ALLERGY: **Penicillin** (childhood rash)

Code status: **Full Code** | Fall risk: HIGH

History: Kidney disease (stage 4), diabetes, weak heart (heart failure)

Family: Wife **Eleanor Kessler** (72, health care proxy); daughter Diane Ruiz (15 minutes away)

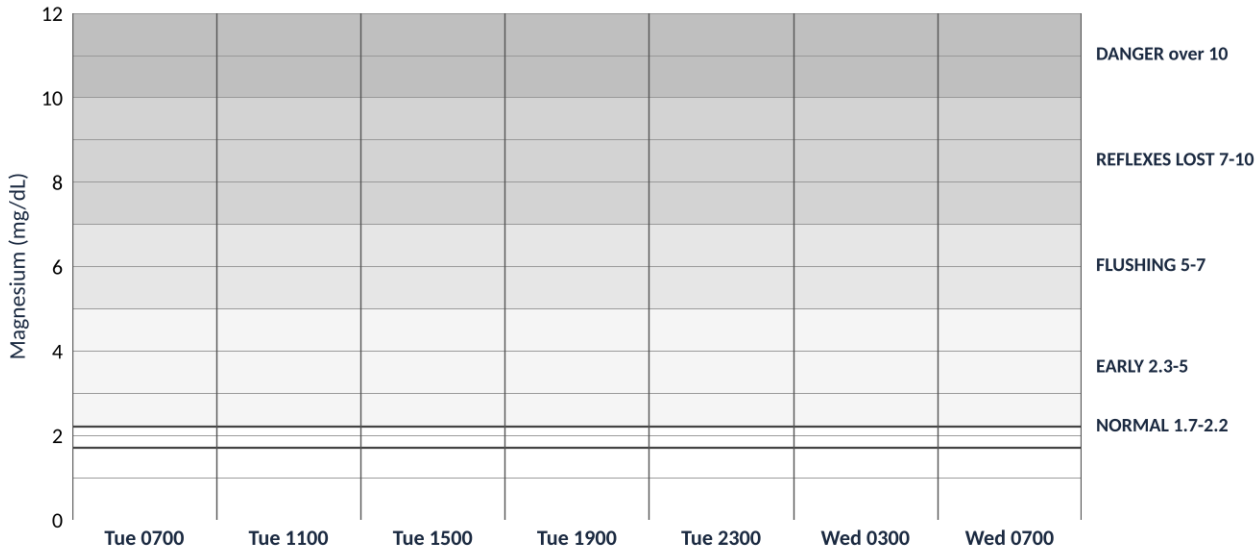
Care team: Hospitalist Dr. D. Okafor (0700-1900); night hospitalist Dr. J. Lindgren; nephrology Dr. P. Raman; ICU outreach Dr. H. Moreno; pharmacist L. Ferreira

MAGNESIUM IN ONE MINUTE

- Normal magnesium is **1.7 to 2.2 mg/dL**. In this lab, anything over **5.0** is a critical value.
- Magnesium leaves the body through the **kidneys**. If the kidneys are failing, magnesium builds up.
- High magnesium slows the **nerves, muscles, breathing and heart**. The bands on the strip show what you might see. Trust your exam as much as the number.

TEAM TREND STRIP: MAGNESIUM

one mark per result



At each checkpoint, write the new magnesium level on your page, then **mark an X** in that checkpoint's column at the right height. Two new results? Mark both. Connect the dots as you go. The charge nurse of the round makes sure it is done.

Use this space for a quick team note (for example, what makes the line go up or down):

Team notes on the trend

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

HOW YOU EARN YOUR 20 POINTS

4 categories | 3 levels each

This is a team grade. A complete, careful, team-minded effort earns full credit. Nothing here is a trick.

Category and points	FULL credit	GOOD (partial)	GETTING THERE
Teamwork and charge-nurse rotation **5 points**	**5.** Every section has a charge nurse named, everyone was charge nurse at least once, and the team worked together at every checkpoint.	**3 to 4.** The rotation was followed most of the time. Most teammates took part.	**1 to 2.** The rotation was skipped, or one person did most of the work.
Clinical thinking at each checkpoint **6 points**	**6.** Every checkpoint has vitals, meds, results and changes filled in, with real numbers from the chart and a reason.	**4 to 5.** Most checkpoints are filled in with numbers. One or two are thin.	**1 to 3.** Many boxes are blank or have one-word answers.
Safety, priorities and SBAR **5 points**	**5.** At every checkpoint, 3 priorities in order, a complete SBAR (all four parts) and a next step that keeps Ray safe.	**3 to 4.** Most checkpoints have priorities and an SBAR. A part is sometimes missing.	**1 to 2.** Priorities or SBAR are missing at many checkpoints.
Completeness and reflection **4 points**	**4.** All boxes tried, the magnesium trend strip is marked, and each person handed in the reflection page.	**2 to 3.** Most boxes tried. The strip or a reflection is missing.	**1.** Many blanks. Little reflection.

WHAT WE LOOK FOR ON EVERY CHECKPOINT PAGE

- A **charge nurse is named** and the team has decided who looks at what.
- The boxes are filled in with **real numbers from the chart** (vitals, labs) and a short reason.
- Three **priorities in order** and a complete **SBAR** (Situation, Background, Assessment, Recommendation).
- The **magnesium level** is written and marked on the team strip.
- A **prediction** for the next 4 hours. A good guess with a reason earns credit, even if it turns out wrong.

AT THE END

- Each person hands in a short **reflection** (3 boxes) and a quick **peer teamwork check** on the last page.
- Answers that differ from the instructor's are fine if they are thoughtful and tied to Ray's chart.

Category	Points possible	Points earned (instructor)
Teamwork and charge-nurse rotation	5	
Clinical thinking at each checkpoint	6	
Safety, priorities and SBAR	5	
Completeness and reflection	4	
TOTAL	20	

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

0700 | Tuesday

Checkpoint 1 of 6

Charge nurse for this stop: _____ **Nurse focus:** ☐ meds and treatments ☐ results and monitor ☐ everyone heard ☐ SBAR done

SITUATION It is **Tuesday 0700** on 5 East, room 5E-14. Ray Kessler, 74, came up from the ED about ten minutes ago after four days of vomiting, and he was hard to wake this morning.

The ED found a **very high magnesium (7.8; normal is 1.7 to 2.2)** and a creatinine of 4.9. They gave calcium gluconate and a saline bolus and placed a Foley. His wife Eleanor is holding a paper bag of medicines from home that nobody has looked at yet.

Time	Temp F	HR	BP (MAP)	RR	SpO2 / oxygen	Pain	Rhythm on the monitor	Glucose
0425 prev	97.2	46	82/46 (58)	14	95% RA	-	Sinus brady (ECG 0430)	148
0700 NOW								

Vitals and what I noticed *read the monitor, then look at Ray*

Current meds and treatments *running, due, or should stop?*

New results and what they mean for his diagnosis

What changed since the last checkpoint?

MAGNESIUM (mg/dL) new level(s): _____ and _____

Compared with last time: ☐ up ☐ down ☐ same

Marked on the team strip ☐

Our top 3 priorities *in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

What we will do next

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask every nurse for one thing, then decide; note who looked at what*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

1100 | Tuesday

Checkpoint 2 of 6

Charge nurse for this stop: _____ **Nurse focus:** ☐ meds and treatments ☐ results and monitor ☐ everyone heard ☐ SBAR done

SITUATION It is **1100**. Since 0700 Ray has had another saline bolus, maintenance saline and IV furosemide (a water pill) at 0915. The pharmacist went through Eleanor's bag and the home products were stopped.

The 0900 labs are back: **magnesium 8.7**, up from 7.8. Eleanor stepped out to call their daughter. You walk in to check on Ray.

Time	Temp F	HR	BP (MAP)	RR	SpO2 / oxygen	Pain	Rhythm on the monitor	Glucose
0700 prev	97.6	52	96/56 (69)	14	94% RA	2/10	Sinus brady with first-degree AV block (PR 0.26 s)	132
1100 NOW								

Vitals and what I noticed *read the monitor, then look at Ray*

Current meds and treatments *running, due, or should stop?*

New results and what they mean for his diagnosis

What changed since the last checkpoint?

MAGNESIUM (mg/dL) new level(s): _____ and _____

Compared with last time: ☐ up ☐ down ☐ same

Marked on the team strip ☐

Our top 3 priorities *in order*

1.

2.

3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

What we will do next

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask every nurse for one thing, then decide; note who looked at what*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

1500 | Tuesday

Checkpoint 3 of 6

Charge nurse for this stop: _____ **Nurse focus:** ☐ meds and treatments ☐ results and monitor ☐ everyone heard ☐ SBAR done

SITUATION It is **1500**. The rapid response team came at 1115 and Ray has had calcium gluconate (1 g at 1120, then 2 g doses ordered). He made only 70 mL of urine in four hours. The 1315 labs: **magnesium 10.9**, potassium 5.8, creatinine 5.3. The 1330 ECG shows HR 38 and a wider QRS (148 ms).

Dr. Raman decided on **emergency dialysis**. Eleanor gave phone consent at 1350, the neck catheter went in at 1440 and the x-ray is clear. **Dialysis starts now.** ICU outreach says Ray can stay on 5 East with a 1:1 nurse.

Time	Temp F	HR	BP (MAP)	RR	SpO2 / oxygen	Pain	Rhythm on the monitor	Glucose
1100 prev	97.4	44	84/48 (60)	10	91% RA	0/10	Sinus brady, long PR (0.30 s)	104
1500 NOW								

Vitals and what I noticed *read the monitor, then look at Ray*

Current meds and treatments *running, due, or should stop?*

New results and what they mean for his diagnosis

What changed since the last checkpoint?

MAGNESIUM (mg/dL) new level(s): _____ and _____

Compared with last time: ☐ up ☐ down ☐ same

Marked on the team strip ☐

Our top 3 priorities *in order*

1.

2.

3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

Charge nurse notes *ask every nurse for one thing, then decide; note who looked at what*

Keep going on the next page (the same time block).

1500 | Tuesday

Dialysis is starting. Same team, same charge nurse.

Checkpoint 3 of 6, page 2

Before dialysis starts: what must be ready and checked?

While dialysis runs: what do we watch, and how often?

When do we call for help? Our call numbers and who we call

What we will do next

Prediction for the next 4 hours *what do we think will happen?*

Anything else the charge nurse wants to record

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

1900 | Tuesday

Checkpoint 4 of 6

Charge nurse for this stop: _____ **Nurse focus:** ☐ meds and treatments ☐ results and monitor ☐ everyone heard ☐ SBAR done

SITUATION It is **1900**. Dialysis ran from 1500 to 1800 and took off 500 mL of fluid. At 1630 his blood pressure dipped to 82/46 and the dialysis nurse gave 250 mL of saline. His magnesium was 7.2 at 1700 and **4.6** after dialysis (1800).

Ray is awake and breathing 16 times a minute. **Your team rolls straight into the night shift** as the day team gives report. Look him over.

Time	Temp F	HR	BP (MAP)	RR	SpO2 / oxygen	Pain	Rhythm on the monitor	Glucose
1500 prev	97.2	46	88/50 (63)	9	92% on 4 L NC	CPOT	Sinus brady, long PR (0.32 s), wide QRS (0.14 s), tall T waves	88
1900 NOW								

Vitals and what I noticed *read the monitor, then look at Ray*

Current meds and treatments *running, due, or should stop?*

New results and what they mean for his diagnosis

What changed since the last checkpoint?

MAGNESIUM (mg/dL) new level(s): _____ and _____

Compared with last time: ☐ up ☐ down ☐ same

Marked on the team strip ☐

Our top 3 priorities *in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

What we will do next

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask every nurse for one thing, then decide; note who looked at what*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

2300 | Tuesday night

Checkpoint 5 of 6

Charge nurse for this stop: _____ **Nurse focus:** ☐ meds and treatments ☐ results and monitor ☐ everyone heard ☐ SBAR done

SITUATION It is **2300**. D10W (sugar water) has run since 1930 at 40 mL/h and Ray's sugars have been 98, 106 and 118. At 2100 his **magnesium was 5.6**, up from 4.6, and his calcium was 7.9.

Eleanor went home at 2100. Since 2230 Ray has been restless and is pulling at his lines. You go in.

Time	Temp F	HR	BP (MAP)	RR	SpO2 / oxygen	Pain	Rhythm on the monitor	Glucose
1900 prev	97.6	66	104/60 (75)	16	96% RA	3/10	Normal sinus rhythm 66, PR 0.20 s	54 (at 1905)
2300 NOW								

Vitals and what I noticed *read the monitor, then look at Ray*

Current meds and treatments *running, due, or should stop?*

New results and what they mean for his diagnosis

What changed since the last checkpoint?

MAGNESIUM (mg/dL) new level(s): _____ and _____

Compared with last time: ☐ up ☐ down ☐ same

Marked on the team strip ☐

Our top 3 priorities *in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

What we will do next

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask every nurse for one thing, then decide; note who looked at what*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

0300 | Wednesday

Checkpoint 6 of 6

Charge nurse for this stop: _____ **Nurse focus:** ☐ meds and treatments ☐ results and monitor ☐ everyone heard ☐ SBAR done

SITUATION It is **0300 Wednesday**. Ray slept a little. The 0100 magnesium came back **6.4** (4.6 after dialysis, 5.6 at 2100) and calcium is 8.1. The D10W stopped at 0130 and his glucose is 126.

Ray has had no stool in four days and his belly looks full. He mumbles something about "the pink milk".

Time	Temp F	HR	BP (MAP)	RR	SpO2 / oxygen	Pain	Rhythm on the monitor	Glucose
2300 prev	98.2	62	112/64 (80)	16	96% RA	3/10	Sinus rhythm 62, PR 0.22 s	118
0300 NOW								

Vitals and what I noticed *read the monitor, then look at Ray*

Current meds and treatments *running, due, or should stop?*

New results and what they mean for his diagnosis

What changed since the last checkpoint?

MAGNESIUM (mg/dL) new level(s): _____ and _____

Compared with last time: ☐ up ☐ down ☐ same

Marked on the team strip ☐

Our top 3 priorities *in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

What we will do next

Prediction for the next 4 hours *what do we think will happen?*

Charge nurse notes *ask every nurse for one thing, then decide; note who looked at what*

Turn in with a paperclip (no staples). Write in dark pen. Do not turn the page until the instructor advances the clock.

0700 | Wednesday

Final check

Charge nurse for this stop: _____ **Nurse focus:** ☐ meds and treatments ☐ results and monitor ☐ everyone heard ☐ SBAR done

SITUATION It is **Wednesday 0700**. After the bisacodyl suppository Ray had a large stool at 0345 and slept. The 0500 magnesium is **5.3** (down from 6.4) and the creatinine is 3.3 (it peaked at 5.3). Urine has recovered to 85 mL/h. Weight is 90.0 kg (88.0 on arrival).

Dr. Raman says no more dialysis for now. The dialysis catheter stays 24 hours just in case. The Foley can come out, and furosemide is due at 0800 for the extra fluid. Eleanor is back at the bedside.

Time	Temp F	HR	BP (MAP)	RR	SpO2 / oxygen	Pain	Rhythm on the monitor	Glucose
0300 prev	98.0	56	104/60 (75)	12	95% RA	4/10	Sinus rhythm 56, PR 0.24 s (long again)	126
0700 NOW								

Vitals and what I noticed

Current meds and treatments *running, due, or should stop?*

New results and what they mean

What changed since 0300 and since arrival?

MAGNESIUM (mg/dL) new level(s): _____ and _____

Compared with last time: ☐ up ☐ down ☐ same

Marked on the team strip ☐

Our top 3 priorities for the day team *in order*

1.
2.
3.

What we will tell the provider (SBAR) *The charge nurse makes the call.*

S Situation	
B Background	
A Assessment	
R Recommend	

What we will teach Ray and Eleanor *three plain-language tips*

The story of Ray's stay in our words *look at the team strip*

Charge nurse notes

Last step: turn to the next page for your individual reflection.

MY REFLECTION AND PEER CHECK

individual

Hand this page in with your own name on it.

Name: _____ Team: _____ Times I was charge nurse: _____

1. Something I learned about Ray's case

2. Something our team did well (and who helped)

3. One thing I would do differently as a charge nurse or as a nurse

PEER TEAMWORK CHECK

Rate up to three teammates (not yourself). Mark one box for each.

Teammate	Did their share	Listened to teammates	Spoke up with good ideas	One kind, useful comment
	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	
	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	
	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	☐ yes ☐ mostly ☐ not yet	

Hand in your reflection with the team packet. Thank you!